



Guideline

Bereavement

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Introduction

Sir Charles Gairdner Osborne Park Health Care Group is committed to providing holistic, compassionate and timely bereavement support to all families. We recognise and respect the unique and diverse needs of individuals and families following the death of their loved one.

Care of the dying, deceased, and those affected by bereavement is a vital component of health and social care.

This guideline applies to the families and carers of patients who have died in Sir Charles Gairdner Hospital (SCGH) or Osborne Park Hospital (OPH).

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☐
Breach professional standards	☐	Misconduct	☐
Breach SCGH Mission & Values	☐	Other:	☐

Bereavement Care Standards

Raising Awareness

Staff working with the dying and bereaved will demonstrate an understanding of death, dying and bereavement. Grief is acknowledged as a natural response to loss, and each person's experience is shaped by their culture, ethnicity, community, beliefs and individual abilities.

Promoting Safe and Effective Care

All staff involved in end-of-life care and bereavement support will provide high quality, safe, sensitive and effective care. This care will be tailored to the individuals' background, beliefs and preferences.

A culture of continuous improvement ensures that interactions with bereaved individuals is supportive and never harmful.

Creating a Supportive Experience

Patients nearing the end of life, and families will be treated with dignity, privacy, and respect. Wherever possible, individuals will be supported to die in their preferred environment, with access to practical, emotional, and spiritual support that reflects their personal needs and values.

Knowledge and Skills

We value a skilled and compassionate workforce. Staff involved in bereavement care will receive ongoing professional development and appropriate supervision to ensure the highest standards of support.

Working together

Effective communication and coordination between hospital staff, General Practitioners, and community services are essential. Integrated care ensures that resources are used efficiently and that the needs of the dying, their families, and carers are met with compassion and professionalism.

Bereavement Care Procedure***Spiritual Care and Chaplaincy Services***

- Families will be asked if they wish to receive support from a chaplain, religious leader, or other spiritual care provider.
- Referrals to Chaplaincy & Spiritual Services may be initiated prior to or at the time of death, particularly if the family has not contacted their own spiritual leader (e.g., parish minister, priest, rabbi, imam).
- SCGH maintains an on-call roster of duty chaplains for emergencies and after-hours deaths, accessible via the hospital switchboard.
- OPH offers part-time chaplaincy services during business hours. For after-hours emergencies, the OPH switchboard can contact the on-call Clinical Nurse Manager.
- Contact details for a wide range of denominations and faith groups are available to chaplains and Clinical Nurse Managers to facilitate specialised spiritual care as required.

Family Support Following Death

All families will be offered:

- Compassionate acknowledgment of the death of their loved one and the opportunity to spend time privately in a suitable setting.
- Bereavement booklet(s) provided at or before the time of death.
- A sympathy card sent within two weeks of the death by the nurse, social worker, or identified team member.
- A follow-up phone call may be made within 12 weeks if a family member is identified as being at risk (refer to Bereavement Guidelines for staff)

Clinical Communication

- Families will be given the opportunity to discuss the care and treatment of their loved one.
- Meetings with the Consultant and relevant members of the treating team will be arranged as needed

Bereavement Documentation

The ward clerk will initiate the [bereavement log sheet](#), which includes:

- The ward clerk will affix the Patient label to the form
- The ward clerk will provide Notification of death to the patient's General Practitioner via fax, accompanied by a cover letter.
- Nursing/social worker or identified team member will document the:
 - date the bereavement booklet was provided
 - date the sympathy card was sent, and booklet provided.
 - follow-up plan if a family member is considered at risk.

- The ward clerk will process the patient as deceased on Webpas; noting at this point all future appointments will be cancelled.
- The ward clerk will forward the finalised bereavement log to Medical Records for scanning.
- A follow-up phone call may be made within 12 weeks if a family member is identified as being at risk; tips for staff on key indicators for people at risk of complex grief can be found in the [Bereavement Guidelines for staff](#).

Staff Support

- Support and assistance are available to all staff through their line manager or the Employee Assistance Program (EAP).
- Education resources are available on the [SCGOPHCG intranet](#).

Related Policies/Documents

- [Bereavement Booklet – information for family and friends](#)
- [Bereavement Guidelines – for staff](#)
- [Bereavement Log Sheet](#)
- [SCGOPHCG Bereavement Intranet page](#)

Authorisation & Version Control

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Applicable Standards	NSQHS Standards Std 1: Clinical Governance  Std 2: Partnering with Consumers  Std 5: Comprehensive Care 				
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